# ICU Admission Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

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| **Case ID** | SYN-0010 | **Age / Sex** | 63 / M |
| **Setting** | Intensive Care Unit | **Primary dx** | Hypoxic-ischemic brain injury after OHCA |

## Chief complaint / reason for admission

Out-of-hospital cardiac arrest with return of spontaneous circulation (ROSC) after 22 minutes.

## History of present illness

Collapsed at a shopping center. Bystander CPR started at 2 minutes, AED shock delivered at 7 min (ventricular fibrillation). EMS: 4 further shocks, adrenaline 3 mg, amiodarone 300 mg, ROSC at 22 min. Intubated in the field. Transported to hospital with targeted temperature management started.

## Past history / medications

Prior anterior STEMI 6 years ago, stent to LAD. Hypertension. Aspirin, bisoprolol, ramipril, atorvastatin.

## Examination

Intubated and ventilated. HR 92, MAP 74 on low-dose noradrenaline. Pupils 4 mm, sluggish. Corneal reflex absent on the left, weak on the right. No spontaneous movement. Myoclonic jerks twice during rewarming.

## Glasgow Coma Scale

No eye opening. Verbal not testable (intubated, scored 1T). Motor: no response to pain on admission; extension (decerebrate) at 72 h.

| GCS on ICU admission (sedation off) | Score | Response |
|---|---|---|
| Eye opening (E) | 1 | None |
| Verbal (V) | 1T | Intubated (T) |
| Motor (M) | 1 | None |
| **TOTAL GCS** | | **3T** |

| GCS at 72 h (off sedation, normothermia) | Score | Response |
|---|---|---|
| Eye opening (E) | 1 | None |
| Verbal (V) | 1T | Intubated (T) |
| Motor (M) | 2 | Extension (decerebrate) |
| **TOTAL GCS** | | **4T** |

## Investigations

ECG: ST elevation anterior leads, coronary angiography showed acute in-stent thrombosis of LAD treated with PCI. CT head: loss of gray-white differentiation. EEG: burst suppression pattern. NSE 78 ug/L at 48 h. MRI at day 5 pending.

## Assessment

Hypoxic-ischemic encephalopathy after prolonged cardiac arrest with poor prognostic indicators (GCS 3T, extension, myoclonus, high NSE, EEG pattern). Multimodal prognostication in progress.

## Plan / disposition

Continue ICU supportive care, avoid hypotension and fever, repeat neurological assessment at 5-7 days. Family meeting scheduled to discuss prognosis and goals of care. Decisions about withdrawal of life-sustaining therapy deferred until multimodal assessment complete.